

Aging and People with Disabilities

Number: APD-PT-25-023

Authorized by: Jane-ellen Weidanz

Issue date: 12/12/2025

Subject: Rate Methodology Change for AFH and RCF Settings

Transmitting (check the box that best applies)

- ☐ New policy ☒ Policy change ☐ Policy clarification ☐ Executive letter
☐ Administrative Rule ☐ Manual update ☐ Other:

Applies to (check all that apply):

- | | |
|--|--|
| ☐ All ODHS employees | ☐ County DD Program Managers |
| ☒ Aging and People with Disabilities | ☐ Developmental Disabilities Services |
| ☒ Area Agencies on Aging: -- Type B -- | ☐ Self-Sufficiency Programs |
| ☐ Child Welfare Division | ☐ Support Service Brokerage Directors |
| ☐ Children's Intensive In Home Services | ☐ Stabilization and Crisis Unit |
| ☐ Children's Residential Services | ☐ Other (please specify): Enter Text |

Policy/rule title:

Policy/rule numbers:

Release number:

Effective date: January 1, 2026

Expiration date:

References:

Web address:

Discussion/interpretation

The Oregon Legislature has directed Oregon Department of Human Services (ODHS) to adopt a new model for calculating provider rates based on the individualized needs of residents.

Effective January 1, 2026, Adult Foster Homes (AFH) and Residential Care Facilities (RCFs) will have a rate methodology change. The new rate methodology does not apply to AFHs or RCFs with Specific Needs Contracts or to Assisted Living Facilities (ALFs). With this change, the new rate methodology will replace the current methodology of base plus add-ons. **Oregon ACCESS (OA) will automatically calculate and apply the new methodology to assessments: no additional actions are required.**

An overview of these changes is provided in a training module, which may be viewed here:

<https://wd5.myworkday.com/oregon/learning/course/aa4d259b6b3c10015a61e97763c90000?type=9882927d138b100019b928e75843018d>

A webinar regarding this change is also under development and will be announced in a separate transmittal once it is finalized.

The Office of Aging and People with Disabilities (APD) aims to support a person-centered approach, increase transparency and reduce the need for hourly rate exceptions. These changes are also designed to maintain quality of care for the individuals we serve.

OA and the Mainframe systems will be updated to reflect these changes. A separate transmittal will provide screenshots of the system changes and other technical information.

Additional information regarding the methodology, rate changes (including where to find the rate change on the 512), and exceptions process will be sent to providers as a provider alert.

Point Calculation

Assessment points:

Each component of the CA/PS assessment will be given a number of points based upon the assessed need level as follows:

- One (1) point for Independent
- Two (2) points for Minimal Assist
- Three (3) points for Assist
- Five (5) points for Substantial Assist
- Six (6) points for Full Assist

In most instances, if the assessment response has three choices (A, B, C), the result is either Independent, Assist, or Full Assist. If the assessment response has four choices (A, B, C, D), the result is either Independent, Minimal Assist, Substantial Assist, or Full Assist.

Additional Cognition Points:

Individuals assessed as a Full Assist in Self-Preservation, Decision Making, and/or Make Self Understood will receive an additional 10 points. Individuals assessed as a Substantial or Full Assist in Challenging Behaviors will receive an additional 20 points. The maximum additional points cannot exceed 20 points.

Treatment Points:

Treatments that have triggered the [Complex Medical Add-on](#) will also have points, based upon frequency. If the qualifying treatment is provided at least weekly, one point will be provided. If the qualifying treatment is provided at least daily, three points will be provided.

Additional Treatment information: Individuals who require sliding scale insulin must have this treatment and the “Medications requiring skilled assessment/judgment/monitoring” treatment indicated in the Treatment section in CA/PS. In addition, adding treatments to an assessment that is in “Completed” or “Administrative” status will not calculate the additional points.

Points Calculated into Tiers

The points described above will be added up to determine which tier the individual is assessed at. A new assessment is not required to generate the new tier. The systems will automatically convert to the new rate structure on 1/1/2026.

Assessed Levels	Score Ranges	Tier
Low	0-40	Tier 1
Moderate	41-55	Tier 2
Medium High	56-82	Tier 3
High	83-106	Tier 4
Very High	107+	Tier 5

Each tier will have an established rate for January 2026. For AFHs, the tiers fund one caregiver 24 hours per day (which includes at 16 hours of awake care) plus additional daily staff hours.

Tier	AFH Rate	AFH Additional Staff Hours	RCF Rate
Tier 1	\$2,332	0	\$2,863
Tier 2	\$3,327	2	\$3,421
Tier 3	\$3,863	4	\$3,979
Tier 4	\$5,916	8	\$4,537
Tier 5	\$7,773	10	\$5,172

The calculation of these tiers will be done automatically by ODHS systems.

“Hold Harmless” Provision for Current Exceptions

Individuals who currently have an approved exception may have a total approved rate (base plus add-on rate and the exception rate) that is higher or lower than the tier rate for 2026.

If the total approved rate is **lower** than the tier rate, the exception will automatically end, and the individual will be at the assessed tier rate. For example, if an individual is currently at a total approved rate of \$7,000 and is at Tier five, the rate for this individual will be \$7,773, starting January 2026.

If the total approved rate is **higher** than the tier rate, the individual will continue with that total approved rate from 2025. For example, if an individual is currently at a total approved rate of \$12,000 and is at Tier five, the rate for this individual will continue into January 2026 at the \$12,000 rate. This rate will be maintained until either an assessment shows a lower tier, the individual moves out of the home, or during the renewal process, it is determined that the provider is not providing the exceptional care through payroll records.

Providers with rates under this hold harmless provision will need to submit payroll records, staffing plans, and form 514A to renew their exception under this rate. The payroll records must reflect that the provider is staffing for:

- The resident's tier,
- The resident's exception, and
- Other residents who have an exception (this includes staffing for the tier and exception hours)

If these records are not submitted by the time the exception expires, the hold harmless provision will end. The provider will receive the consumer-assessed tier going forward unless another exception is approved.

CBC New Exceptions Process:

AFH exception requests will only be granted when:

- An individual's needs exceed the caregiving hours as funded by a resident's assessed tier, and

- All other individuals receiving Medicaid LTSS are staffed according to the tier daily staffing requirements and any approved exceptions. This is in addition to a caregiver who is on duty 24 hours a day (i.e. the owner or the resident manager)

Example

A provider has four residents with the following tiers and exceptions:

Resident	Tier	Caregiving Hours funding by each Tier	Exceptional Hours	Total Hours
A	5	10	2	12
B	4	8	0	8
C	2	2	0	2
D	5	10	4	14

If the provider wants to admit a fifth resident at Tier three and request an exception, they must demonstrate—through payroll records—that they are staffing at least one caregiver on duty 24 hours a day and providing a total of 36 caregiving hours per day.

An exception may be approved for 90-180 days and may require payroll records from the provider if they have a history of inadequate staffing or if the exception was approved before a resident moved into the AFH.

The department may reduce or terminate an exception when the individual's needs no longer justify the hours funded by the exception and/or when the provider fails to submit the required documentation or does not provide the additional staffing as authorized.

Process for requesting an exception or requesting a renewal of an exception:

The following must be submitted with each exception request by the AFH provider:

- The Department's AFH Exceptions Calculator – The calculator will be provided on CM Tools, under the Long-Term Care Facilities section.
- A current 514 – An updated 514 will be provided on CM tools, under the Long-Term Care Facilities section. The version on the forms server will be updated later.
- The following payroll documentation:
 - Pay stubs showing hours worked and taxes withheld
 - Documentation of rate of pay
 - Receipts of payroll and employer tax submission to the Internal Revenue Service (IRS) and the Oregon Department of Revenue (DOR)
 - Time sheets verifying when caregivers work designated shifts. These timesheets must be signed by the caregiver.
 - The department may request additional documentation if the department has concerns about the validity of the documents submitted.

Note: Local office staff should ensure that providers are including each of the payroll records indicated above, however staff are not expected to verify the records in detail.

Local/branch action required

Local office staff must ensure the following steps are completed before submitting an exception request:

- All documentation and information required by this transmittal has been submitted by the provider.

- All provider-submitted documentation is date-stamped if received via fax or in-person.
- A new CA/PS assessment must be conducted if the exception request indicates increased needs that conflict with the current assessment. This requirement may be temporarily waived in urgent situations or when the individual is discharging from a hospital. In such cases, the exception may be approved for up to 90 days, and a CA/PS assessment must be completed before the exception can be renewed.
- A supervisor or lead worker has reviewed the documentation and CA/PS assessment and agrees the exception is justified and the CA/PS assessment has been updated as needed.

Note: Local offices may deny exception requests. In such cases:

- The provider must receive a written explanation for the denial.
- If denying a renewal of an existing exception:
 - The provider must receive a 10-day written notice before the exception ends, including justification for the denial.
 - Central Office must be notified to ensure the rate is stopped at the appropriate time. This may require extending the rate to allow for the full 10-day notice.
 - If the denial is due to insufficient staffing, the provider must have been given 14 days to submit an updated staffing plan before the denial.

Additional responsibilities:

- Send all required documents to Central Office for a determination.
- If Central Office requests additional documentation (e.g., payroll records), local office staff must communicate these requirements to the provider.

- Once Central Office approves or denies the exception, local office staff must notify the provider of the decision.

Exceptions submitted prior to January 1st, 2026, will still be processed and will be approved with two different rates. If approved, one rate will reflect the exception hours and rate from the effective date to December 31st, 2025. The second rate will show the rate and any exceptional hours approved after January 1st, 2026.

Central office action required

Regarding AFH Exceptions, Central Office staff will:

- Review exception requests submitted by local offices.
- Request additional documentation from local offices as needed.
- Review payroll documentation.
- Provide written determinations to local staff for communication to providers.

Reviewed by impacted parties? ☒ Yes ☐ No

If yes, reviewed by: Operations and Internal Review

Filing instructions:

If you have any questions, please contact:

Contact(s): CBC Exceptions Team

Email: APD.CBCExceptions@odhsoha.oregon.gov